

The Protein Floor

The one number to get right while you're losing — and how to actually hit it.

FROM A PHYSICIAN ASSISTANT WHO'S BEEN ON A GLP-1 HERSELF

When you're eating significantly less, the food that's left has to work harder. This isn't a diet rule and it isn't about being good — it's arithmetic. Protein is the thing that gets crowded out first, because it's the one that takes effort.

WHY PROTEIN STOPS BEING OPTIONAL

Losing weight is never purely fat. Some of what comes off is *lean tissue* — muscle. How much depends heavily on what you're eating and whether you're using your muscles at all. Higher-protein diets during weight loss are generally associated with retaining more lean mass than lower-protein ones.

Muscle isn't decorative. It's what lets you carry things, get off the floor, and stay steady on stairs at seventy.

Why this matters more in your forties and fifties

Hormonal shifts during perimenopause and menopause — particularly declining estrogen — are linked to increased fat storage and reduced muscle mass. Muscle loss accelerates in this window on its own. Two things pulling in the same direction is why protein moves from “nice idea” to “the thing to get right.” To be clear about what I'm not saying: protein does not fix hormones, and nothing here does. This is about what you can control.

The unexpected bonus

Protein is the most satiating macronutrient — it slows digestion and influences hunger-related hormones. The thing you're eating for your muscles is also the thing that keeps you from being feral at 3pm. Usually the useful thing and the pleasant thing are different things.

Your floor, and how to actually hit it

STEP ONE: FIND YOUR NUMBER

If you are...	Aim for (per kg body weight, daily)
Mostly sedentary	~0.8 g/kg
Physically active	1.2–1.6 g/kg
Losing weight, or holding onto muscle	1.6–2.2 g/kg

Your weight in pounds ÷ 2.2 = kilograms. Multiply by your number. (The old RDA was the minimum to prevent deficiency — a different question than keeping muscle while losing.)

STEP TWO: SPREAD IT OUT

Evenly distributing protein across meals improves muscle protein synthesis, particularly in women over 40. Your body can't bank it. Coffee for breakfast plus a big dinner does *not* equal the same total spread across the day. **Roughly even across three meals. That's the whole technique.**

STEP THREE: FRONT-LOAD IT

On a GLP-1 your appetite is often most cooperative early. By evening, food can be genuinely unappealing — so take the protein when the taking is good. I am a bad cook; this is not a skill problem, it's a sequencing one. Zero-talent options:

- Eggs, any way you can manage them
- Greek yogurt or cottage cheese
- A protein shake — no cooking, no opinions
- Leftover anything from dinner. Breakfast food is a convention, not a rule.

What to watch for: hitting your number and still feeling weak or wobbly on stairs is worth raising with your provider — not pushing through. And protein alone won't hold muscle: resistance of some kind, a couple of times a week. Carrying groceries counts more than nothing.

Where to go from here

The floor is the point. Not perfection — a number you don't go under, spread across the day, most days.

If you don't know your number

You already have everything you need for it, two pages back: **your weight in pounds ÷ 2.2, times the number on your row of the table.** That is the whole calculation. Write it on something you will see.

If you're hitting it and still feel awful

That's a conversation with your provider, not a reason to try harder. Ask specifically about the target for your body.

If you've never asked anyone what your target should be

Ask. It's a two-minute question — age, kidney function, medications and history all change the answer. A number from a table is a starting point, not a prescription.

I'm a physician assistant, I've been on a GLP-1 myself, and the questions I get most are about side effects — not results.

Come find me: Instagram @diary_of_the_most_okayest_mom — I answer questions there, and the side-effect stuff nobody warns you about is most of what I post.

Educational only. This is not medical advice and it isn't a substitute for your own provider's judgment. Protein targets are general guidelines — individual needs vary with age, muscle mass, health conditions and dietary restrictions; only a licensed provider can personalize them. Results are not guaranteed and individual results may vary.

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